

Aquablation (Waterjet Prostate Surgery)

WARWIKI

Patient Information · Image-guided waterjet surgery for an enlarged prostate

Aquablation is a newer surgery for an enlarged prostate (BPH) that blocks urine flow. A strong **jet of salt water**, guided by an ultrasound picture, removes the blocking tissue through the urethra. There is no cut on the skin. Flow results are similar to TURP, and men are **more likely to keep their ejaculation**.

About This Procedure

The surgeon uses a scope and a live **ultrasound** picture of your prostate to map exactly which tissue to remove. A **robot-guided waterjet** then removes that tissue. The waterjet itself uses no heat. Afterward, the surgeon controls bleeding with a catheter balloon and, when needed, a short step that uses heat.

Benefits and Risks in Numbers

1 in 10

men lost ejaculation in the main study, compared with about 1 in 3 after TURP

19 in 20

did not need another prostate operation within 5 years

2 in 100

had new urine leakage at 1 year (large prostates)

Numbers are averages from published studies. Your own risk may differ; ask your team.

Is It Right for Me?

- It is used for small to large prostates, and it can treat a middle lobe.
- **Bleeding is the main early risk.** It is higher in large prostates, and some men need a blood transfusion.
- It is newer, so data beyond 5 years are still growing.

Learn the Terms

Aquablation

Removing prostate tissue with a waterjet to treat BPH.

BPH

An enlarged prostate that is not cancer.

Ultrasound

A live picture made with sound waves. It lets the surgeon map what to treat.

Middle lobe

A part of the prostate that can bulge into the bladder and block flow.

Retrograde ejaculation

Semen goes back into the bladder instead of out.

Transfusion

Getting donated blood through a vein.

Catheter

A soft tube that drains the bladder for a short time after surgery.

How is it different from TURP? TURP trims tissue with an electric wire loop. Aquablation removes it with a waterjet guided by ultrasound. Urine flow improves about the same with both. Aquablation is more likely to **keep ejaculation**. The trade-offs: it is newer, and bleeding can be more of a concern.

First day or two

Catheter. Most men go home the next day.

First few weeks

Pink urine, burning and urgency. Drink fluids. No straining or heavy lifting.

Follow-up visit

Your team checks your flow and how well you empty.

How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection first.
- You may have a scan to measure your prostate.
- Arrange a ride home.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Bleeding or burning that gets worse instead of better

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- Your urine looks like red wine or has clots, or the catheter stops draining
- You cannot reach the office and are worried

What Happens

- 1 You get **general or spinal anesthesia**, so you feel nothing, and antibiotics.
- 2 Ultrasound maps the prostate. The surgeon plans the area to treat.
- 3 The waterjet removes the tissue. Then bleeding is controlled.
- 4 A catheter is placed. It often comes out within 1–2 days.

Three Things to Remember

1. Aquablation removes blocking prostate tissue with a waterjet guided by ultrasound.
2. Flow results are close to TURP, with a better chance of keeping ejaculation.
3. Bleeding is the main early risk. Ask your team about your own risk.